

DISCHARGE SUMMARY

Patient: David Thomas MRN: SYN-DC-1003 DOB: 1965-07-09
Admission Date: 2026-03-18 Discharge Date: 2026-03-25
Attending Physician: Dr. M. Fernandez, Cardiology

PRIMARY DIAGNOSIS

Non-ST elevation myocardial infarction (NSTEMI), status post percutaneous coronary intervention (PCI) with drug-eluting stent to the left anterior descending artery.

HOSPITAL COURSE

Patient presented with acute substernal chest pain radiating to the left arm, associated with diaphoresis. Initial troponin was elevated at 2.4 ng/mL, rising to 8.1 ng/mL on repeat. ECG showed no acute ST elevation but T-wave inversions in the anterior leads. Cardiac catheterization revealed 85% stenosis of the proximal LAD, treated with a single drug-eluting stent. Patient remained hemodynamically stable post-procedure. Started on dual antiplatelet therapy and high-intensity statin. Echocardiogram showed mildly reduced ejection fraction of 45%.

DISCHARGE MEDICATIONS

1. Aspirin 81mg PO daily, lifelong.
2. Clopidogrel 75mg PO daily for 12 months.
3. Atorvastatin 80mg PO nightly.
4. Metoprolol succinate 50mg PO daily.
5. Lisinopril 5mg PO daily.

FOLLOW-UP INSTRUCTIONS

Cardiology follow-up in 1 week. Cardiac rehabilitation referral provided. Repeat echocardiogram in 3 months to reassess ejection fraction. Strict adherence to dual antiplatelet therapy is critical to prevent stent thrombosis; do not discontinue without cardiology consultation.